



Publication trends in Benzodiazepine and SSRI Co-Prescription Versus Monotherapy: Effects on Adherence and Anxiety Relief

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BACKGROUND

Selective serotonin reuptake inhibitors (SSRIs) are first-line treatments for anxiety disorders but require 2-4 weeks to achieve therapeutic effects[1]. Benzodiazepines are often co-prescribed to manage this period[2]. However, while intended for short-term use, real-world prescribing patterns suggest that some patients transition into longer-term use[3,4], raising concerns about safety and treatment trajectories[5,6].

Current research on this practice is fragmented across clinical trials, observational audits, and animal models. While clinical rationale for co-prescription is well-established[2,7], there is a lack of consensus on its impact on long-term adherence and SSRI persistence[8,9]. This diversity in research focus makes it difficult to determine which areas of the field are most emphasized and where gaps – particularly in neurobiological and molecular mechanisms – may remain.

A bibliometric approach allows for a systematic evaluation of these trends. By mapping the distribution of study types and influential literature, this study identifies the structural organization of the field and highlights areas that may be underrepresented in the current evidence base.

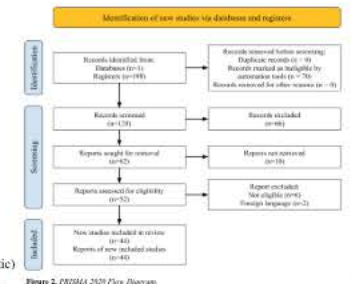
OBJECTIVE

- Analyze trends and structural patterns in benzodiazepine-SSRI co-prescription research
- Quantify the distribution of study types and research domains
- Examine representation of symptom relief, duration, and adherence outcomes
- Identify gaps and imbalances in the current literature

METHODS

- **Data Source:** Web of Science Core Collection (through 2026)
- **Screening & selection**
 - Preferred Reporting Items for Systematic Reviews and Meta-analyses (PRISMA)
 - 11 reviewers screened records
 - Metadata extracted
 - first author
 - senior author
 - publication year
 - article title
 - Journal
 - citation count
- **Analysis**
 - Bibliometric analysis performed using RStudio Biblioshiny
 - Figures were created using Excel and RStudio
 - Studied categorized by
 - Study type (clinical, animal, mechanistic)
 - Outcome focus (short-term, long-term, adherence/discontinuation)
 - Study design (RCT, observational/retrospective, review)

Keywords: "benzodiazepine" AND "SSRI" AND "anxiety"



Inclusion criteria	Exclusion criteria
Peer-reviewed articles and reviews	Duplicate record
Relevant to co-prescription or monotherapy outcomes	Not in the English language
English language	Irrelevant to research objective

Figure 3. Inclusion and Exclusion Criteria.

RESULTS

Key Contributor Trends

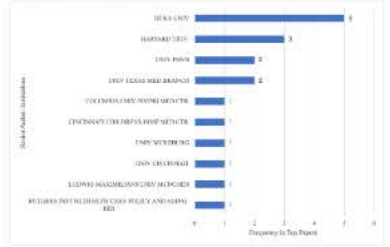


Figure 4. Senior Author Institutions (R50 Multiple Publications)



Figure 5. Senior Author Journals (R50 Multiple Publications)

Methodological Approaches

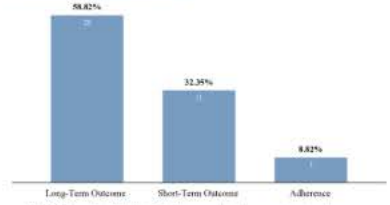


Figure 7. Outcome Focus Distribution. The literature emphasizes long-term outcomes while underrepresenting adherence – the central rationale for co-prescription as a short-term 'bridge' therapy.

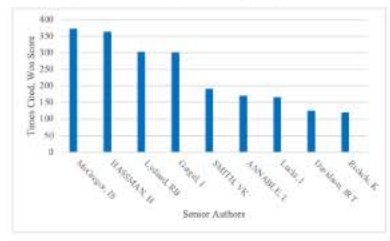


Figure 6. Senior Authors with Multiple Publications in the Top 100 Most Cited Papers

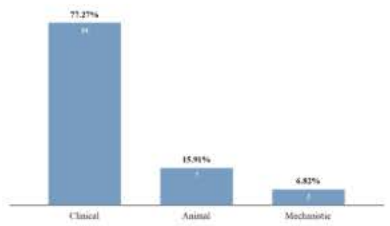


Figure 8. Study Type Distribution. The distribution indicates a strong emphasis on clinical outcomes, with relatively limited exploration of underlying biological mechanisms.

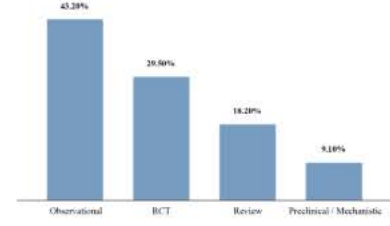


Figure 9. Study Design Distribution. The predominance of real-world study designs reflects a focus on prescribing patterns and long-term experience, while highlighting a relative gap in high-quality causal evidence.

Key Study Findings

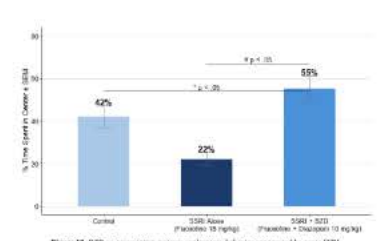


Figure 10. SSRI Co-prescription reduces explanatory behavior suppressed by acute SSRI administration. (1/4 - Barlett Study)

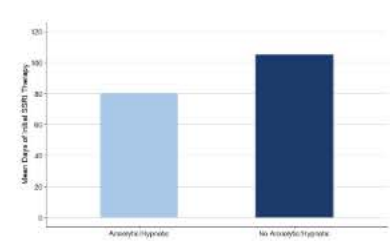


Figure 11. SSRI Persistence and Discontinuation (1/4 - Barlett Study)

DISCUSSION

- Benzodiazepine-SSRI co-prescription research is predominantly clinical and emphasizes long-term treatment trajectories.
 - Co-prescription is intended as a short-term strategy, but the literature primarily focuses on long-term outcomes.
- Adherence outcomes are underrepresented, despite being a primary rationale for co-prescription.
 - This highlights a mismatch between clinical justification and research emphasis.
- Observational studies predominate over randomized controlled trials.
 - The reliance on real-world data may limit causal interpretation of treatment effectiveness.

CONCLUSION

- Co-prescription is often transformed from a short term 'bridge' to long-term maintenance without established discontinuation protocols
- Adherence, a key metric for co-prescription, remains underrepresented in current research
- Co-prescription literature is predominantly clinical and indicates long-term continuation outcomes
- Greater emphasis on longitudinal and mechanistic studies is required for improved alignment with clinical practice
- Future research should monitor co-prescription side effects, control for biological moderators, and evaluate alternative co-prescription pairings

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